

Policy

Care Coordination in Mental Health Services

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Functional subgroup	Mental Health
Summary	Care coordination is the process of helping people experiencing mental health concerns to access a range of different services to support their recovery journey. The Care Coordinator is the allocated member of the multidisciplinary team (MDT) who is responsible for facilitating integrated mental health care and support, including collaboration between different services and people.
Policy owner	Executive Director, Safety, Quality and Consumer Engagement
Contact	
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Impact Statement Declaration (ISD)	Consultation through Aboriginal community is reflected in this policy for appropriateness and cultural considerations for positive health impacts on the Aboriginal community. (ISD Record ID)
Status	Active

About this document

This document outlines the key elements of care coordination and describes the roles and responsibilities of all mental health clinicians who are involved in coordinating care for people accessing [mental health services within the East Metropolitan Health Service \(EMHS\)](#). This document provides overarching governance for all EMHS mental health services, including statewide services, and should be read in conjunction with relevant legislation, standards, and policies.

Care Coordination in mental health services is to be provided in alignment with the EMHS values of kindness, respect, collaboration, excellence, integrity, and accountability.

1. Background

Care coordination is a person centred, assessment based, recovery-focused, interdisciplinary approach to integrating health care and support. Individual needs and preferences are assessed, a comprehensive Treatment Support and Discharge Plan (TSDP) is collaboratively developed, and services are coordinated and monitored by an allocated Care Coordinator. Evidence indicates that care coordination improves experience of care, promotes a recovery approach, improves quality of life, and delivers better health outcomes.

All people accessing mental health services within EMHS will have access to care coordination through appropriate services that support their care from the point of entry or re-entry to discharge. Entry to community mental health services will be based on referral, triage, and assessment. The Care Coordinator (or allocated clinician within the multidisciplinary team; MDT) will ensure that appropriate contact and coordination are maintained throughout any inpatient admission to support a smooth transition between care settings.

EMHS is committed to providing a safe and supportive working environment to EMHS staff, who undertake their practice in all setting types including inpatient and community mental health services and the residences of people who are receiving care. EMHS also acknowledges the duty of care to provide optimal support and risk mitigation strategies to staff where there is an awareness of pre-identified or anticipated risk factors, to support safety of all.

Refer to [MP0099/18 Community Mental Health Status Assessment: Role of MH Clinician](#)

Refer to [EMHS Community/home visiting Policy](#)

1.1. Key Definitions

Term	Definition
Care Coordinator	The allocated member of the multidisciplinary team (MDT) who has responsibility for coordinating, facilitating, and integrating mental health treatment, care and support for a person accessing mental health care and their support people.

	• The care coordinator is allocated based on a 'best fit' between the needs of the person accessing care and Care Coordinator knowledge, skills, and experience. • Care coordinator provides a single point of contact for people accessing care, their support people, and other services. • A nominated member of the MDT will be allocated the role of Care Coordinator during periods of absence of the primary Care Coordinator[^] [^] The primary Care Coordinator should advise the person accessing care of planned leave and who the allocated Care Coordinator will be during this time (if known).
The Multidisciplinary Team (MDT)	• A team of mental health staff usually comprised of a variation of disciplines including psychiatry, psychology, nursing, occupational therapy, social work, pharmacy, peer support workers, Aboriginal Health Liaison Officer (AHLO), welfare officer and other relevant groups who collaborate in the assessment, planning and delivery of care. • The MDT support the Care Coordinator in taking a holistic approach to the needs of the person accessing care, including physical, social, emotional, and spiritual wellbeing, relevant to their different disciplines. • The MDT provide specialised input to ensure that the person accessing care is provided with appropriate assessment, evidence-based interventions, assertive care, and discharge planning.
Warm Referrals	• Facilitating a warm referral involves the referring clinician providing the person accessing care with information about the new service and (with consent of the person accessing care) contacting the service they are referring to and making an appointment. This also allows the clinician to provide the new service with information about the person accessing care, to support a smooth entry into new services. • Warm referrals bridge gaps between services, minimising duplication and reducing the onus on people accessing care and carers to navigate complex systems and provide the same information to multiple providers.
Recovery	• Recovery refers to the journey that a person experiences in relation to their mental health. • Recovery is a concept which can incorporate hope, self-determination, and a meaningful life to the person; it can have different meanings to different people.

	It is recognised that people may experience barriers and challenges on their mental health journey, but this does not mean there is not progress and growth. It is important to recognise not only challenges, but positive aspects of mental health journeys.
Support people	Support people includes carers, family members, nominated people and other people identified by a person accessing care as a personal support person.
Relevant service providers	- Relevant service providers are those outside of the treating health service who provide physical or mental health care or psychosocial supports, including, but not limited to: - Non-Government Organisations - Community Controlled Organisations - Aboriginal Community Controlled Organisations - General Practitioners - Private psychiatric hostels

2. Guiding principles

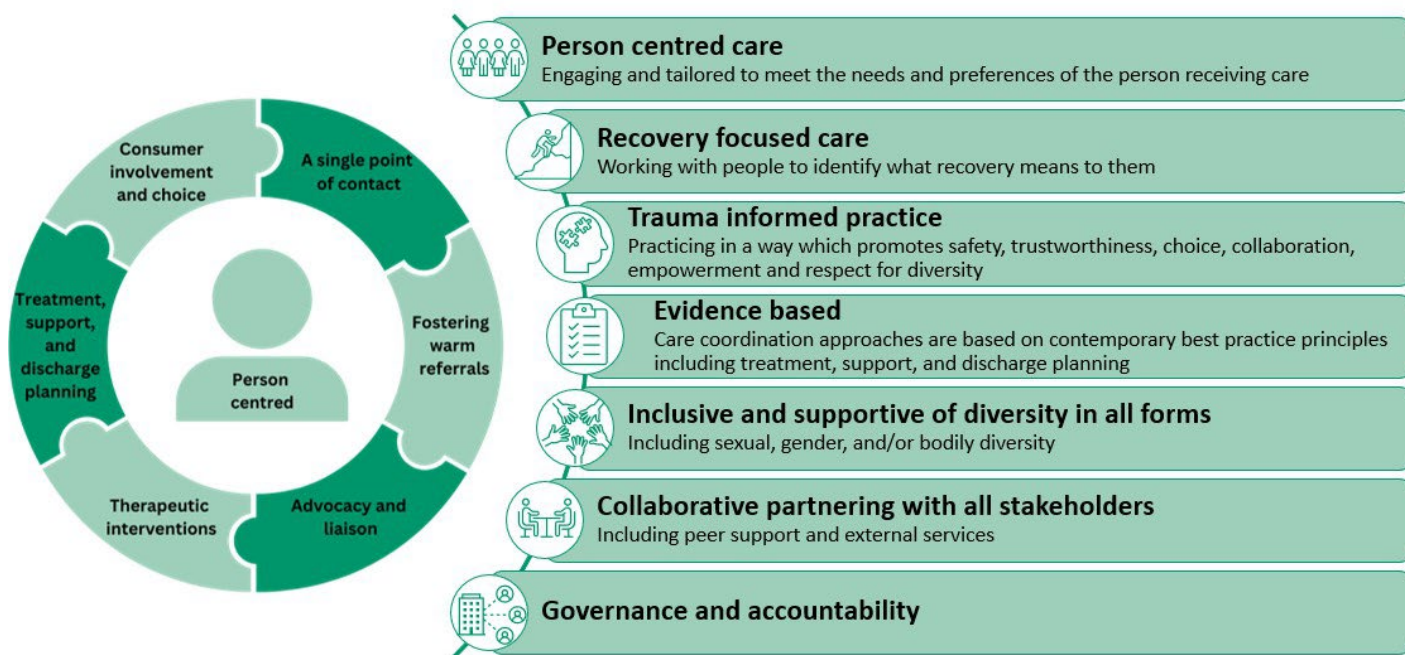
The following principles underpin Care Coordination:

- Person centered care
- [Recovery focused care and language](#)
- Trauma informed practice
- Socially inclusive and supportive of diversity in all its forms including cultural, sexual, gender and/or bodily diversity.
- Engaging and tailored to meet the needs and preferences of the person accessing care.
- Evidence-based
- Collaborative partnering with all stakeholders.
- Governance, accountability, and scope of practice according to service model of care

3. EMHS Model for Care Coordination

The EMHS Model for Care Coordination encompasses the roles and responsibilities of the Care Coordinator and the guiding principles for the delivery of care coordination.

Taking these roles, responsibilities and principles into account, person centered care remains at the forefront of all care coordination activities. Person-centered care is about providing the services and support that are right for each person, taking into account their unique needs, circumstances, preferences, and goals. Person-centered care focuses on what each person needs and what works for them. Service providers must work in partnership with people receiving care, and their support people, to understand their personal preferences, establish trust and mutual respect, and plan and make decisions about care.



4. Referral and Access

At point of referral, a clinician will be allocated to support the triage and entry process to the service. This clinician will be allocated to provide support within the appropriate time frame as per the EMHS [Acute Response Mental Health Emergencies / Crisis Policy](#).

Once the required referral / entry information and appropriate consent have been obtained, the person will be activated into the relevant program, or provided support into other relevant identified services, and a Care Coordinator allocated as per the process outlined in section 6 of this policy.

5. Allocation of a Care Coordinator

A Care Coordinator will be allocated at the program specific intake meeting via MDT discussion once a person has agreed to be activated into the service, and at other points as clinically indicated. The role of Care Coordinator will be undertaken by an allocated member of the multidisciplinary team.

The selection of a Care Coordinator will be based on the following:

- Considering needs, complexity, and potential requirements, including Community Treatment Orders, for the person receiving care.
- Ensuring the consideration of relevant background factors, including culture, diversity, sexual, gender and/or bodily diversity of the person receiving care.
- The Care Coordinator's professional qualification/s, knowledge, skills, and experience aligns with the needs of the person receiving care and supports their journey.
- Seeking to achieve a 'best fit' between the needs of the person receiving care and the Care Coordinator's knowledge, skills, and experience. Identifying a 'best fit' Care Coordinator supports in building and maintaining the therapeutic relationship.
- The current caseload allocation of the Care Coordinator considering acuity levels and volume.

- In the event the needs of the person receiving care changes significantly, or they have expressly requested a change to the allocated Care Coordinator, this should be considered through escalation and discussion within the MDT.
- Where a person re-enters the service and is accepted by the same treating team, the previously allocated Care Coordinator should be considered, in addition to the above criteria.
- In the event the allocated Care Coordinator is unavailable, a secondary Care Coordinator should be allocated.

6. Care Coordination and the Multidisciplinary Team

The MDT is a key vehicle for collaboration in the assessment, planning and delivery of mental health services. It enables both core and specialist assessments and interventions to be provided in an integrated manner, in collaboration with the person receiving care, their support people and other relevant service providers. Whilst there will be a single allocated Care Coordinator, it is recognised that the task of providing comprehensive services is beyond the capacity of one professional.

The MDT will usually comprise clinicians from psychiatry, psychology, nursing, occupational therapy, social work, pharmacy, Peer Support Workers (where applicable) and other relevant groups. Aboriginal Mental Health Workers or Aboriginal Liaison Officers should also be considered where appropriate and available. The MDT may also include representatives from Non-Government Organisations, who work in partnership with EMHS mental health services.

There are both shared tasks and specialist tasks in the coordination of care, and these are performed across the MDT membership and within specific disciplines, respectively.

The role of the MDT in care coordination is to, collaboratively:

- Participate in the decision regarding the allocation of the Care Coordinator.
- Participate in discussion and decision regarding level of assertiveness required to engage and involve the person in their treatment and support, particularly where engagement is an issue.
- Participate in regularly scheduled and ad hoc clinical reviews, led by the Care Coordinator, to ensure that specialist skills and advice are maximised to meet the requirements, including escalation if needs change.
- Participate in discharge planning.
- Participate in the re-allocation of the Care Coordinator if it is indicated that an alternate skill set is required for an extended duration of time to best support the person who is receiving care.

7. Care Coordinator – role and responsibilities

The Care Coordinator, in collaboration with the MDT is responsible for tailoring care and support to meet the specific and unique needs of the person receiving care and their support people. This includes facilitating and integrating mental health assessment, treatment and support that is evidence based and aligned with the choice of the person receiving care, alongside all relevant legislations, standards, and policies.

Care Coordinators should undertake education around key roles and responsibilities of care coordination and take part in other relevant education and training activities as per site requirements.

Key functions of the Care Coordinator include:

7.1. Promoting engagement, involvement, and choice

The Care Coordinator promotes engagement and choice at all possible times for the person who is accessing the service, to support them to be actively involved in their care. The Care Coordinator will support the person to be actively involved in their own care and treatment by:

- Creating an effective therapeutic relationship with the person, emphasising trust and collaboration to support their journey in a way that is meaningful.
- Holistic assessment including mental, physical, psycho-social, cultural, and sexual, gender and/or bodily diversity needs.
- Assisting the person in identifying their personal goals, individual strengths, and perceived obstacles that should be considered in their Treatment, Support, and Discharge Plan (TSDP).
- Considering choices, options, and preferences to promote and facilitate a recovery approach.
- Providing an inclusive service and ensuring appropriate actions are taken to engage other services, e.g., interpreters, linking with cultural groups or other groups for needs related to sexual, gender and/or bodily diversity.
- Ensuring respect for privacy, dignity, and independence (including requirements for obtaining Consent for Treatment and Release of Information).
- Having clear and transparent discussions regarding roles and responsibilities with people who are receiving care and their support people. This includes expectations of both the service and the person receiving care in relation to treatment, support, and discharge planning.
- Maintaining regular contact through assertive outreach at agreed intervals.
- Escalating issues to the MDT for discussion and decision regarding assertion required to engage and involve the person in their treatment, care, and support.

7.2. Acting as primary contact within the community mental health service

The Care Coordinator acts as the primary contact between the person receiving care and the mental health service, as well as the primary contact with the MDT on behalf of the person. This function is undertaken by:

- Providing a single point of contact for the person accessing mental health services and their support people.
- Enabling and fostering partnerships through identifying all support people that are involved in the care of the person.
- Engaging in discussion and planning regarding goals on entry to, and discharge from, an inpatient service.
- Maintaining contact and coordination during any inpatient (mental or general health) admission, as per service models of care. This may include in-reach to support the person on the ward or attending the inpatient MDT meeting.

- Facilitating discussions regarding care and discharge planning within the Community Mental Health Service, with the MDT and at Clinical Review Meetings, including facilitating input from the person receiving care, their support people, GP, and other primary health care professionals. This may include escalation of issues / risks to the MDT and obtaining an expert opinion when required.
- Ensuring completion of all required documentation, as per the [WA Health Statewide Standardised Clinical Documentation for Mental Health](#) (SSCD).
- Ensuring collaborative review of the individual care plan with the person at least every three months, to facilitate ongoing delivery of treatment, care, and support in line with the SSCD, Office of the Chief Psychiatrist Standards for Clinical Care, and the National Safety and Quality Health Service (NSQHS) Standards (second edition).

7.3. Brokering of services

Facilitating warm referrals to any additional required services that support psycho-social, cultural, environmental, and sexual, gender and/or bodily diversity needs is a key element of holistic care planning. The Care Coordinator undertakes this by:

- Referring, or coordinating referrals, to other services as required, including, but not limited to:
 - Non-Government Organisation's (NGOs),
 - Community Managed Organisation's (CMOs) for psycho-social support,
 - Traditional Healers for Aboriginal people,
 - National Disability Insurance Scheme (NDIS) to access psycho-social services,
 - Government agencies for family, housing, and financial support.
- Participating in shared assessments with additional services, where possible and with the consent of the person accessing care, to ensure collaborative service partnerships starts from point of referral.
- Supporting the person accessing care with their physical health care needs including linking with their General Practitioner (GP), including Aboriginal Community Controlled Health Services (ACCHSs), and attending medical appointments and other treatments/therapies when required/scheduled.
- Assisting and supporting the person receiving care to access other health services when indicated (e.g., vision, hearing, dental, podiatry, sexual health services).
- Assisting the person receiving care in obtaining legal representation when needed.
- Supporting follow-through with all outside referrals for benefits and services.

7.4. Advocacy and liaison functions

Care Coordinators, where applicable and supported by the person accessing care, may assume the role of advocate or liaison to support achieving independent living.

Advocacy may be undertaken by:

- Working with and supporting the Mental Health Advocacy Service (MHAS), if the person is an identified person under the Mental Health Act 2014, ensuring their rights are maintained.
- Supporting the person receiving care in navigating housing, justice and other services/agencies where required

- Helping to safeguard the rights of the person in line with the [Mental Health Act 2014 Charter of Mental Health Care Principles](#). Further information can be found via the [Mental Health Advocacy Service](#).

Liaison roles may be undertaken by:

- Effective engagement in clinical handover processes to ensure explicit transfer of professional responsibility and accountability for some or all aspects of care, to another person or professional group on a temporary or permanent basis, as per the [WA Department of Health Clinical Handover Policy](#).
- Ensuring timely communication and liaison with services/individuals involved in the care of the person, including GP/ACCHSs, multidisciplinary team members, inpatient teams (when relevant) and other agencies.
- Ensuring communication and liaison with support people.
- Acting as a liaison in mental or general health services when the person receiving care may require hospital admission.

7.5. Care planning

Development, and regular review, of the TSDP must be in collaboration with the person receiving care and their support people (where possible), in line with the [Statewide Standardised Clinical Documentation for Mental Health](#). Information in the TSDP will include, but not be limited to:

- The person's needs, goals, current support, and treatment
- Views of the consumer, family, carer and/or PSP
- Safety planning and risk mitigation
- Phase of care and National Outcome and Casemix Collection (NOCC) measures
- Crisis and relapse prevention plans and outcome measures
- Physical health care needs and requirements (e.g., metabolic screening, Liverpool University Neuroleptic Side Effect Rating Scale [LUNSERS], ASSIST)
- Role and responsibilities of the person receiving care and the Care Coordinator
- Date of next clinical review
- Discharge planning and identification of planned discharge date

Best practice guidelines in relation to treatment options should be considered, as appropriate and per service models of care. Site based Quick Reference Guides can provide further information regarding care planning.

- [Royal Perth Bentley Group Care Coordination Quick Reference Guide](#)
- [Armadale Kalamunda Group Care Coordination Quick Reference Guide](#)

7.6. Therapeutic interventions

The Care Coordinator, in partnership with the MDT and acting within their defined scope of practice, will utilise evidence-based practice to provide a range of therapeutic interventions that are recovery-oriented and tailored to meet the needs of the person receiving care.

Therapeutic interventions must be delivered in alignment with the [Chief Psychiatrist's Standards for Clinical Care](#) and the [National framework for recovery-oriented mental health services](#).

Therapeutic support should be delivered in all phases of care, as supported by site models of care and appropriate Standard Operating Procedures (SOPs).

Note: The Care Coordinator must communicate to the MDT any change in clinical presentation that requires escalation or review of care or intervention.

Interventions will be based on comprehensive multidisciplinary assessment and collaborative, person-centered, care planning and will include, where applicable:

- Responsive and evidence-based treatments, as appropriate and per service models of care
- Trauma informed and recovery focused care.
- Collaboration with the person receiving care (and their support people if applicable) to identify possible coping strategies to assist with their individual situation.
- Monitoring effects of, and adherence to, TSDP including prescribed medication. This includes psychoeducation relative to any treatment plan.
- Promoting and supporting life skills development to empower independence.
- Psychoeducation and support regarding mental health concerns for the person receiving care and their support people.

7.7. Discharge planning

Discharge planning is integral to reducing risk and ensuring seamless transition of care (including to external agencies). Discharge planning should be commenced at the earliest opportunity after entry (or re-entry) to the service, and a Planned Discharge Date (PDD) discussed with the person accessing the service, to allow for development of a person-centered journey. The treating psychiatrist, Care Coordinator, and MDT undertake discharge planning by:

- Establishing expectations with the person receiving care and their support people regarding the potential length of treatment within the community program at commencement with the service, and at appropriate times throughout treatment, support, and discharge planning.
- Collaborating with the person receiving care and their support people regarding requirements for ongoing care and options, based on discussion with the MDT and relevant ongoing services where applicable (e.g., shared care where appropriate to treatment provision and service model of care).
- Providing information to the person receiving care about confidentiality and sharing information and required consent.
- Development of a TSDP in collaboration with the person receiving care and their support people, in line with phase of care and PDD.
- Development of a comprehensive Safety Plan with the person receiving care and their support people, as per the [Safety Planning for Mental Health Consumers policy](#).
- Phone contact and handover to ongoing service providers (including GPs) to support provision of warm referrals.
- Discussing and providing written information regarding follow-up appointments, ongoing service providers and emergency care contact details to the person and their support people.

- Discussing and providing written information regarding re-entry to the service to the person and their support people, ensuring there is a clear pathway if required.
- Ensuring timely preparation and communication of discharge information, as per the [EMHS Discharge Communication \(Written\) Policy](#) and site-based procedures for discharge.

8. Care Coordination and the Inpatient Team

Where a person receiving care is admitted to an inpatient unit, the Care Coordinator must work collaboratively with the inpatient treating team to support the provision of care which is in alignment with the care coordination model of care, where possible.

Care Coordination Transfer of Care Quick Reference Guides outline inpatient admission to discharge guidelines and responsibilities.

- [Royal Perth Bentley Group Care Coordination Transfer of Care Quick Reference Guide - Bentley, City East, Wungen Kartup Mental Health Services](#)
- [Royal Perth Bentley Group Care Coordination Transfer of Care Quick Reference Guide - RPBG Midland Community Mental Health Service & St John of God Public & Private Hospitals](#)

8.1. Referral / Admission

Where the Care Coordinator is the referring clinician, they must work with the inpatient team to facilitate the referral and admission process to support a smooth transfer of care. This includes the Care Coordinator providing handover to the inpatient treating team and engaging in discharge planning from admission. Documentation must be completed as per [SSCD Triage to Discharge Pathway](#).

Where the Care Coordinator is not the referring clinician, the inpatient treating team must seek out and engage the Care Coordinator.

Where the person receiving care has an identified General Practitioner (GP) or engages with other relevant service providers, the inpatient treating team must ensure notification of admission and any changes to treatment planning.

8.2. Inpatient admission

During the inpatient admission, the Care Coordinator must maintain engagement with the person receiving care and their treatment plan. This may include attending ward rounds, family meetings, and engaging in treatment planning.

The inpatient treating team must ensure engagement with the Care Coordinator throughout the admission, including timely communication to support attendance at relevant meetings. Where no Care Coordinator is identified, but will be required following discharge, a referral must be made to Community Mental Health Services for allocation of a Care Coordinator prior to discharge.

8.3. Discharge

The inpatient treating team must engage with the person receiving care, their support people and the Care Coordinator in discharge planning and coordination of ongoing care and treatment following discharge. Timely communication with the Community Mental Health team is required ensure their capacity to participate in discharge and care planning for the person accessing the service.

Where a person has been referred for Community Mental Health follow up, the inpatient MDT must provide follow up appointment information (where possible), crisis and emergency contact and pathway information prior to discharge.

The inpatient team must alert the relevant Community Mental Health team prior to discharge and provide handover to the Care Coordinator to support smooth transition of care, including relevant documentation as per [SSCD Triage to Discharge Pathway](#).

Where the person receiving care has an identified General Practitioner (GP) or engages with other relevant service providers, the inpatient treating team must ensure notification of discharge and any changes to treatment planning.

8.4. 7 day follow up

7 day follow up should be completed as per site policies and procedures, relative to the inpatient setting.

Note: If concerns are identified from 7 day follow up, they must be communicated to the Care Coordinator / MDT by phone in a timely manner and documented in the medical record and PSOLIS.

9. Compliance monitoring and legislative penalties

9.1. Compliance monitoring

Each EMHS site or Executive Director(s) are to ensure compliance with this policy.

Service level Safety and Quality Committees should monitor and report through to the peak clinical governance body of each EMHS site with the following information:

- Number (n) and percentage (%) of people activated within the program:
 - who have an allocated Care Coordinator
 - with clinical review every three months
 - with completed TSDP.
 - with a RAMP completed in required time period.
 - with an identified GP liaison service event.
 - with evidence of support people (family/carer) involvement, as per service events
- Number (n) and percentage (%) of completed National Outcome and Casemix Collection (NOCC) measures.
- Number of mandatory service event items per program.
- Compliance with 7-day follow up as per site operational procedures.
- Compliments and complaints

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Annually EMHS report confirming compliance and implementation of the policy will be submitted to a relevant AEG committee/ subcommittee.

9.2. Legislative obligations

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

10. Legislation.

- [Mental Health Act WA 2014](#)
- [Carers Recognition Act 2004](#)

10.1. WA Health Policies

- [WA Health Clinical Risk Management Guidelines. A best practice guide \(2019\)](#)
- [MP0099/18 Community Mental Health Status Assessment: Role of MH Clinician](#)
- [MP0175/22 WA Health Consent to Treatment Policy](#)
- [MP0130/20 WA Health Complaints Management Policy](#)
- [MP 0015/16 WA Health Information Access Use and Disclosure Policy](#)
- [MP0051/17 Western Australian \(WA\) Language Services Policy](#)
- [MP 0155/21 WA Health State-wide Standardised Clinical Documentation for Mental Health Services](#)
- **EMHS Policies**
 - [EMHS Acute Response in a Mental Health Emergency / Crisis Policy.](#)
 - [EMHS Mental Health Advocates Policy.](#)
 - [EMHS Discharge Communication Policy.](#)
 - [EMHS Community / Home Visiting Policy.](#)
 - [EMHS Cultural considerations when caring for Aboriginal People.](#)
 - [EMHS Rights of Patients in Mental Health Policy](#)
 - [EMHS Rights of Carers and Other Personal Support Persons Policy](#)

11. Additional references

- [WA Health Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist - Policy for Public Mental Health Services \(2018\)](#)
- [Chief Psychiatrist's Standards for Clinical Care \(OCP, 2015\)](#)
- [National Standards for Mental Health Services \(Commonwealth of Australia, 2010\)](#)
- [National Safety and Quality Health Service Standards \(second edition\) \(ACSQHC, 2017\)](#)
- [Armadale Kalamunda Group – Care Coordination](#)
- [Royal Perth Bentley Group – Care Coordination](#)

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- [Treatment Support and Discharge Planning: Information Gathering in a Community Setting. Armadale Kalamunda Group Mental Health Service.](#)
- Australian Health Ministers Council (2013) [A national framework for recovery-oriented mental health services.](#) Policy and Theory 2013.
- Dixon, L, Holoshitz, Y, Nossel, I (2016). Treatment engagement of individuals experiencing mental illness: review and update. *World Psychiatry* 15: p13-20.
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- Mental Health Coordinating Council (2011). [Care Coordination Literature Review and Discussion Paper. Mental Health Service Coordination and the Community Managed Mental Health Sector in the Context of the National Health and Hospitals Reform Agenda.](#)
- [WA Health Strategic Intent 2015-2020](#)
- [WA Youth Health Policy 2018-2023](#)
- [WA Primary Health Care Strategy](#)
- [All Paths Lead to a Home: WA 10 Year Strategy on Homelessness 2020- 2030](#)
- [National Drug Strategy 2017-2026](#)